

ENT Handoff / Referral Note

Compiled from inpatient records — Sir H. N. Reliance Foundation Hospital, Mumbai · Generated 29 April 2026

Patient: Saroj Agarwal · **Sex/Age:** Female / 63 y · **DOB:** 27 Aug 1962 · **Patient ID:** 0010663186 · **Treating Oncologist:** Dr. Darshit Shah

Reason for ENT review: Status post left vocal cord lateralization (26 Feb 2026) for bilateral vocal cord palsy. Persistent aspiration on barium swallow (7 Apr 2026) and reduced laryngeal sensation on FEES; currently on pleasure pureed diet with Ryle's tube feeding. Repeat FEES planned after next chemotherapy cycle. Background of metastatic Ca uterus with FDG-avid left cervical adenopathy on PET-CT.

1 · Background — oncology & presentation

- **Primary diagnosis:** Carcinoma of uterus (ICD-10 C54) — operated 3 years prior; current admission was for metastatic recurrence.
- **Index admission:** 20 Feb → 8 Apr 2026 (47-day inpatient stay) for breathlessness, cough, bilateral pedal oedema, left pleural effusion (ICD in situ since 12 Feb 2026); pleural fluid showed atypical cells (?large-cell morphology).
- **Staging — WB FDG PET-CT (21 Feb 2026):** Moderate to high-grade FDG-avid **left cervical lymph nodes (levels II, III, IV, V) — partly necrotic**; abdominal adenopathy. Report recommended *histopathological correlation from left supraclavicular node*. (Potentially explains the recurrent-laryngeal-nerve compromise — not explicitly stated in record.)
- **Current oncology plan:** Chemoimmunotherapy with Paclitaxel + Carboplatin + Dostarlimab. Last cycle: **C3 D8 on 13 Apr 2026** (most recent encounter).

2 · ENT issue — bilateral vocal cord palsy

Date	Event
24 Feb 2026	Stridor noted; ENT consult. Flexible nasopharyngolaryngoscopy: bilateral vocal cords in paramedian position with restricted mobility and minimal glottic chink . Close airway observation; tracheostomy considered if needed.
25 Feb 2026	Desaturation episode → MET call. Repeat ENT eval: restricted cord mobility (left > right). MDT discussion with family — choice between tracheostomy vs. vocal cord lateralization ; lateralization chosen.
26 Feb 2026	Left vocal cord lateralization under general anaesthesia. Pre-op echo: severe PAH (PASP ~72 mmHg), LVEF 60%, mild RV dilatation. Intra-op desaturation requiring intubation ; post-op shifted to ICU on mechanical ventilation; ABG showed hypoxaemia.
Early Mar (3–4 Mar)	Phonation and breathing improved post-procedure. Weaned to nasal prongs.
9 Mar 2026	Episode of acute bronchospasm attributed to microaspiration — managed with nebulisation, steroids, supportive care; symptoms improved.
18 Mar 2026	FEES #1: improved vocal cord approximation and voice quality; no overt aspiration ; minimal residual feed in valleculae. Adduction exercises continued.
23 Mar 2026	FEES #2: reduced laryngeal sensation → placed on puree diet with Ryle's tube feeding and ongoing speech/swallow therapy.
7 Apr 2026	Barium swallow (post vocal cord lateralisation): free flow oral cavity → stomach; pyriform sinuses and valleculae normal; thin streak of barium into right tracheo-bronchial tree — suggestive of aspiration . Acute desaturation (SpO ₂ 84–85% on room air) followed the study.
8 Apr 2026	Discharged on pleasure pureed diet only , strict safe-swallow precautions, speech therapy continuing.
13 Apr 2026	Daycare for chemotherapy — " <i>Continue oral diet as tolerating with strict safe swallow</i> "; speech-therapy exercises 3×/day.

3 · Current ENT functional status (most recent records)

- **Voice:** Breathy but improving (per speech therapist's review during admission).
- **Airway:** Maintained via lateralized cord; no tracheostomy.
- **Swallow:** Aspiration documented on barium 7 Apr 2026; FEES showed reduced laryngeal sensation. **Pleasure pureed diet only + Ryle's tube.**
- **Plan from oncology team:** repeat FEES after next chemotherapy/immunotherapy cycle.

4 · Comorbidities relevant to ENT (peri-procedural risk)

High anaesthesia / bleeding risk — please consider before any further laryngeal instrumentation.

- **Severe pulmonary hypertension** — PASP 72–91 mmHg on serial echos; RV dilated with systolic dysfunction. On Tadalafil 20 mg HS.
- **Saddle submassive pulmonary embolism** diagnosed 25 Mar 2026 (CTPA). On **Apixaban 5 mg PO BD** (preceded by therapeutic Clexane). Anticoagulation should be planned around any procedure.
- **Polymorphic PVCs / runs of VT** during admission — on **Amiodarone (Cordarone) 200 mg BD** and Mexiletine 150 mg TDS.
- **Anaemia** — Hb 8.3 g/dL (CBC, 13 Apr 2026); RBC 2.82, HCT 25.7 %. (Late-March nadir was Hb 7.3 / Plt $51 \times 10^9/L$ on 29 Mar; recovered by discharge.)
- **Persistent hypoalbuminaemia / hypoproteinemia** — Albumin **2.91 g/dL (27 Mar) → 2.73 g/dL (13 Apr)**; Total Protein 4.74 → 4.63 g/dL (ref 6.4–8.3). Relevant to wound healing if further laryngeal surgery is planned, and to drug binding.
- **Lab — most recent (13 Apr 2026)**: renal function preserved (eGFR > 90, creatinine 0.63 mg/dL); platelets $199 \times 10^9/L$.
- **Sputum cultures (during admission)**: budding yeast / pseudohyphae — antifungals administered.

5 · Current relevant medications

Medication	Indication / note
Apixaban 5 mg PO BD	Anticoagulation for saddle PE — bleeding risk
Amiodarone 200 mg PO BD	Ventricular arrhythmia
Mexiletine 150 mg PO TDS	Ventricular arrhythmia
Tadalafil 20 mg PO HS	Pulmonary hypertension
Spironolactone 50 mg PO OD	Diuresis / RV unloading
Pantoprazole 40 mg PO BD; Ondansetron 8 mg PO TDS	Chemo-related GI prophylaxis
Foracort 1 mg neb 12 hrly; Duolin neb SOS	Bronchospasm prophylaxis (post-microaspiration)
Speech-swallow therapy 3x/day	Ongoing

Specific questions for ENT:

- Is the persisting aspiration on barium (7 Apr) an indication for further intervention (e.g., revision lateralization, medialization, vocal cord injection)?
- Should we proceed with a **repeat FEES sooner** rather than waiting until after the next chemo cycle, given documented microaspiration?
- Given the FDG-avid left cervical levels II–V adenopathy on PET-CT, is an ENT-led **FNAC / core biopsy** of the supraclavicular node feasible, as the radiologist suggested?
- Long-term plan for the Ryle's tube — when can full oral feeds be considered?

Source documents (full PDFs and per-report markdowns are available):

- **DISCHARGE** [report_pdf/2026-04-08_Summary-Discharge_Discharge-Summary.pdf](#) — primary narrative, ENT timeline
- **SWALLOW** [report_pdf/2026-04-07_XRay_X-RAY-Barium-Swallow.pdf](#) — most recent objective swallow study
- **PET-CT** [report_pdf/2026-02-21_CT_WB-FDG-PET-CECT-scan.pdf](#) — left cervical adenopathy
- **DAYCARE** [report_pdf/2026-04-13_Summary-Daycare_Daycare-Discharge-Summary-Chemotherapy.pdf](#) — most recent encounter, current diet/therapy plan
- **PERI-OP** [report_pdf/2026-02-26_XRay_X-Ray-Portable-Chest-AP.pdf](#) — chest film on procedure day
- **CARDIAC** [report_pdf/2026-03-24_ECHO_Echocardiography.pdf](#) — pulmonary hypertension status
- **CTPA** [report_pdf/2026-03-24_CT_CT-Angiography-Pulmonary.pdf](#) — saddle PE
- **LABS** [report_pdf/2026-04-13_Lab-Haem_Complete-Blood-Count.pdf](#), [report_pdf/2026-04-13_Lab-Biochem_Renal_Profile.pdf](#) — most recent

Master date-grouped index: [MASTER_INDEX.md](#). All 188 reports as one file: [ALL_MD_REPORTS.md](#).

This handoff note was assembled from the patient's electronic record (188 reports, 20 Feb – 13 Apr 2026). All facts are sourced from the original PDFs cited above. Please cross-verify against the source PDFs before any clinical decision.